

Why equicenta™ CTM

Evidence-based competitive positioning for field conversations

Key Insight: Lead author Dr. Alicia Bertone authored BOTH the Pro-Stride landmark RCT (2014, n=40) AND the equicenta CTM trial (2026, n=138). Same researcher, 3.5× the horses, real-world SOC comparator instead of saline.

What eCTM Beat: Two-thirds of all SOC injections in the trial (67%) were autologous blood-derived therapies — APS, ACS/IRAP, PRP, and A2M. eCTM outperformed them across every measured outcome with markedly fewer injection reactions.

CLINICAL EVIDENCE COMPARISON

Evidence Quality — eCTM vs. Competitors

Product	Best Study	Design	N	Evidence
equicenta CTM	Bertone 2026	Prospective controlled trial	138	Level I–II
Pro-Stride APS	Bertone 2014 (AJVR)	RCT (blinded, placebo)	40	Level II
IRAP / ACS	Weinberger 2008	Retrospective case series	262	Level III
PRP (general)	2025 PRISMA review	Systematic review	11 studies	Level III–IV
VetStem Adipose MSC	VetStem surveys	Owner survey	66	Level III
RenoVo (amnion)	Duddy 2022	Prospective case series	100	Level III
Hilltop Bio	Whitaker/LSU pilot	Unpublished case series	~10	Level IV–V

COST PER OUTCOME

The True Cost of Treatment

Product	Vet Cost	Owner Cost / Tx	Protocol	Annual to Owner	Evidence
equicenta CTM	\$825	\$1,300–\$2,000	Single injection (most cases)	\$1,300–\$2,000	Prospective trial; 2.72× RTP; 4× safer
Maintain	\$374–\$625	\$561–\$1,250	Single injection, periodic	\$1,122–\$3,750 (2–3×/yr)	Leverages eCTM platform
Pro-Stride APS	\$369/kit (\$1,897/6-pk) + centrifuge	\$1,000–\$1,500	2–3 injection series	\$2,000–\$4,500	RCT (n=40); 14 days
IRAP / ACS	~\$200–\$400/processing	\$1,500–\$3,000 (series)	3–4 injection series	\$1,500–\$3,000+	Uncontrolled
VetStem	\$1,200/kit	\$2,000–\$3,500	Harvest + 48-hr processing	\$2,000–\$3,500+	Retrospective: 77% RTW

Product	Vet Cost	Owner Cost / Tx	Protocol	Annual to Owner	Evidence
Cultured Stem Cells	\$2,000–\$3,000 (harvest + lab grow-out)	\$5,000–\$10,000	Surgical harvest + lab expansion	\$5,000–\$10,000	Limited controlled data
Restigen PRP	\$267/kit (\$1,338/6-pk)	\$600–\$900/injection	2–3 injection series	\$1,200–\$2,700 (2–3×)	Heterogeneous; no RCT
Generic PRP kit	\$50–\$150/kit	\$150–\$300/injection	2–3 injection series	\$300–\$900 (2–3×)	Heterogeneous; no RCT
RenoVo	Est. \$300–\$600	\$500–\$1,200	Single injection	\$500–\$1,200	Uncontrolled (conflicted)
Hilltop Bio	~\$367	\$400–\$1,000	Single injection	\$400–\$1,000	No peer-reviewed pubs
Corticosteroid + HA	\$15–\$50 drug cost	\$200–\$500	Repeat q3–6 mo	\$400–\$2,000	Symptomatic only; risk

VALUE REFRAME

Reframing the Alternatives

equicenta CTM

\$825 vet cost — single injection
One injection in most cases — vs. the 2–3 injections ProStride and PRP typically require. 2.72× odds of return to performance, markedly fewer injection reactions (SOC 4.8–5.9× the odds), sustained to 26 weeks — highest value per treatment dollar.

IRAP Series

\$1,500–\$3,000 — 3–4 injections
No controlled efficacy data. SOC biologics (incl. ACS/IRAP) had markedly more injection reactions in the EPL trial — excess joint reactions were largely from PAAG, but SOC still had 3.4× the odds even excluding PAAG (p<0.03). Higher total cost, higher joint AE risk, weaker evidence — all for a product requiring 24-hr incubation and multiple injections.

VetStem

\$2,000–\$3,500 per cycle
Invasive surgical harvest, 48-hour processing delay, retrospective survey data only.

Repeat PRP / Corticosteroid

\$600–\$2,000/yr ongoing
Palliative only — no structural modification evidence. SOC horses rescued with eCTM matched primary eCTM outcomes (p=0.87). The “cheaper” alternatives often lead to re-treatment injection costs that exceed eCTM.

CLINICAL EVIDENCE

Clinical Highlights

Bottom line: In the largest controlled clinical trial of its kind — 138 horses, 195 conditions, 8 sites, 11 veterinarians — eCTM outperformed real-world standard-of-care biologics across every measured outcome, in a single off-the-shelf injection, with markedly fewer injection reactions.

2.72× return to performance 2.72:1 odds of returning to prior performance level versus standard-of-care biologics (p<0.006).	3× more likely to be pain-free eCTM horses were roughly 3× more likely to be completely pain-free than SOC-treated horses (p<0.001).
Markedly fewer injection reactions SOC biologics had 4.8× the odds of injection reactions on vet review (5.9× central); 3.4× even excluding PAAG (p<0.03).	Durable to 26 weeks eCTM benefits were sustained through 26 weeks, while SOC outcomes declined after the 12-week mark.
One off-the-shelf injection A single thawed 1.5 mL injection in most cases — no harvest, no incubation, no multi-injection series.	First controlled allograft trial in horses The first controlled clinical trial of a tissue allograft in horses — a real-world SOC comparator, not saline.